

A MIXED METHODS EVALUATION OF THE INVOLVEMENT OF NUTRITION ASSISTANTS IN CANCER CARE

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INTRODUCTION

Nutrition Assistants (NAs) are specialised Allied Health Assistants (AHAs) who work under the guidance of dietitians in a clinical setting. (1) An AHA is trained to work within a specific scope of practice, supporting clinicians by carrying out delegated tasks relevant to their area. At Peter MacCallum Cancer Centre (PMCC), NAs have been an integral part of the Nutrition department since 2010. (2) Over the past 16 years, the role has evolved considerably, and the team has doubled in size.

Currently, NAs at PMCC support dietitians by reviewing delegated patients across inpatient and day-patient settings, screening and triaging food allergies, and providing general high energy high protein (HEHP) education to patients and families. They also assist with the provision of Oral Nutrition Supplements (ONS) under dietitian guidance and complete a range of additional administrative tasks.

This evaluation aims to examine the involvement of NAs in cancer care at PMCC, to identify service delivery gaps, ensure responsibilities remain within appropriate scope of practice, and inform improvements to processes, care pathways, and staffing. Findings may also help guide broader adoption of NAs across Australian oncology hospitals.

METHODS

This evaluation will use a mixed-methods design, comprising a quantitative retrospective audit and a set of qualitative cross-sectional surveys. The audit will be conducted to examine the volume and nature of patient reviews undertaken by NAs at PMCC. Data will be compared between two single-year snapshots, 2019 and 2026, to identify trends and changes in workload over time. For each period, data will be collected on:

- The total number of patients reviewed (split by tumour stream)
- The setting of review (inpatient vs day-patient)
- The type of review conducted (e.g., HEHP education, ONS check, oral intake review)
- The outcome of the review (e.g., discharge from NA care, referral to a dietitian)

A set of surveys, consisting of fully open-ended questions, will be distributed to key stakeholders, including all current dietitians and NAs within PMCC, along with a sample of approximately 15 patients who have received NA-led care. These surveys will capture perspectives on the value, effectiveness, and impact of

the role. Survey design will be informed by the Consolidated Framework for Implementation Research (CFIR) to ensure key contextual domains relevant to implementation are captured. (3)

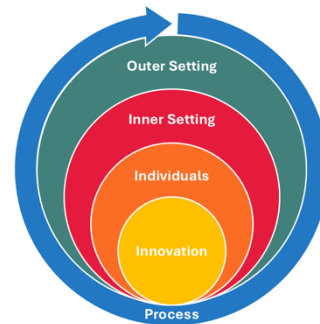


Figure 1. The five domains of the Consolidated Framework for Implementation Research (CFIR), which will inform survey design for this evaluation. Source: <https://cfirguide.org/>

DISCUSSION

One of the most significant developments in the NA role at PMCC in recent years has been their involvement in the care of Autologous Stem Cell Transplant (AuSCT) patients. Following a 2025 care pathway review which identified that many AuSCT patients were well-nourished at baseline and remained so throughout treatment, a shared care model was implemented, allowing well-nourished patients to be managed by NAs with dietitian oversight. (4)

A retrospective audit of this pathway demonstrated positive outcomes, with the majority of patients remaining well-nourished at discharge, alongside high job satisfaction reported by NAs. (4) While these results are promising, they are specific to the AuSCT population. This evaluation seeks to expand on these findings by examining NA involvement across the broader PMCC patient population.

CONCLUSIONS

NAs play an increasingly vital role in cancer care at PMCC. This evaluation will identify service gaps and opportunities across the broader patient population, supporting scope-appropriate growth and the potential expansion of NA models to oncology hospitals in Australia.

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